Defence Guide · Student 1

Student 1: Introduction & Methods Health Anxiety, AI Chatbot Use and Cyberchondria Among Adults in Baghdad

> **Your role.** You open the defence. Your first minute sets the committee's impression of the whole team: slow, clear, confident.

You present	Slides 1–12 (≈ 5 minutes)
You own in the thesis	Chapter 1 Introduction (1.1–1.8), Chapter 2 Method (2.1–2.7), Figures 1–2, Acknowledgment
Your handover line	“That was our methodology. Now my colleague will present what we found.”
Your teammates	Student 2: Results (slides 13–23) · Student 3: Discussion & Conclusions (slides 24–33)

What is in this guide

1. The whole study: two-minute script, core facts, terms, “never say” list (same in all three guides)
2. Your chapter, section by section: what it says, the numbers, the sources, what to watch
3. Your key sources: each reference number in your chapter and what it found
4. Your slides, one by one: what is on screen, your script, timing and traps
5. Questions: on your part, on the whole study, and difficult ones, with model answers
6. Editing your chapter safely and in your own words
7. Your 14-day plan
8. Self-test with answers
9. Checklist for the day before

Part 1 · The whole study (everyone must know this)

> **The committee can ask any of you about any part.** You split the presentation, not the knowledge. Learn this part so well that you can explain the whole study without slides.

The study in two minutes (practise saying this out loud)

"Many people now look up their symptoms online before seeing a doctor, and more and more of them ask AI chatbots like ChatGPT. For some people this searching makes them more worried, not less. That is called cyberchondria, and it is closely linked to health anxiety. No one had studied this in Baghdad, and AI chatbots had hardly been studied next to search engines. We ran an online survey of 300 adults in Baghdad between 17 July 2026 and 11 September 2026, using two established scales: the SHAI-14 for health anxiety and the CSS-12 for cyberchondria. We found that AI chatbots are now the main source of health information: 78.0% ticked them, more than search engines (56.3%). About one in five (19%) had elevated health anxiety. Health anxiety and cyberchondria were moderately linked, r = 0.47, and this held after adjusting for everything else. The link ran mainly through excessive searching and post-search distress, which fits a vicious cycle, although one survey cannot prove direction. We found no reliable evidence that AI itself adds to cyberchondria beyond health anxiety. And searching shapes the consultation: about a third delay care because they feel reassured, and about a third never tell their doctor what they searched. So our main message for clinicians is simple: ask patients, without judgement, what they searched or asked an AI."

Core facts (memorise; examiners test these)

Topic	Fact
Design	Analytical cross-sectional online survey; STROBE and CHERRIES checklists
Who and when	**300** adults, Baghdad, 17 July 2026 – 11 September 2026; convenience + snowball sampling
Sample size	**194** needed for r = 0.20 (α 0.05, 80% power); margin of error ±5.7%
Sample profile	62.3% aged 20–29 · 56.3% female · **44.0%** medical students · 63.3% health background · 21.7% chronic condition · 70.7% answered in Arabic
Tools used	AI chatbots **78.0%** ticked (88.7% incl. named) · search engines 56.3% · medical platforms 18.3% · social media 10.7% · ChatGPT 59.3% · AI without any search engine 41.0%
Motives	convenience 80.0% · "do I need a doctor?" 57.7% · instant reassurance 51.0%
Health anxiety	SHAI mean 12.3 ± 5.8 (0–42) · **19%** elevated (≥ 18; 95% CI 15.0–23.8%) · 0.7% at ≥ 29
Cyberchondria	CSS mean 28.9 ± 8.4 (12–60) · excessiveness highest, compulsion lowest
Reliability	α 0.81 (SHAI), 0.86 (CSS); ω 0.82, 0.87 · CSS-12 4-factor CFA: CFI 0.95, RMSEA 0.070, SRMR 0.053
Arabic vs English	CSS-12 fully (scalar) invariant; SHAI-14 metric invariant; no item DIF after FDR
Main result	**r = 0.47** (95% CI 0.38–0.55); explains 22% of the variance · partial r = 0.48
By facet	excessiveness 0.46 · distress 0.45 · compulsion 0.25 · reassurance 0.24
Regression	R^2 0.030 (demographics) → 0.133 (+ digital use) → 0.317 (+ health anxiety, ΔR^2 18.4%); β = 0.48
Who is anxious	AOR **3.12** per SD of cyberchondria · 3.83 reassurance motive · 0.61 per older age group · AUC 0.83 → 0.79
Pathways	via excessive searching 0.16 (CI 0.11–0.23) · via distress 0.22 (direct → 0.01) · reverse 0.15
Behaviour	delay care when reassured **34.7%** · ignore symptom 30.0% · several doctors 25.3% · urgent care 22.0% · self-medication 21.3%
Doctor relationship	never tell the doctor **31.7%** · always tell 27.0% · trust strained 34.7% · higher cyberchondria → more disclosure (OR 0.57, q 0.012)

AI effect	+5.3 CSS points (inclusive) but +1.5, p = 0.31 (strict) → no reliable independent effect · 32 named-not-ticked · 34 non-users
Robustness	every sensitivity analysis left the main result unchanged (Table 13)

Terms you must be able to explain in one sentence

Term	Plain explanation
Health anxiety	Persistent worry about having or developing a serious illness, kept going by misreading normal body sensations and by reassurance seeking.
Illness anxiety disorder	The DSM-5 diagnosis at the severe end of health anxiety. We did **not** diagnose it; we measured symptoms.
Cyberchondria	Excessive or repeated online health searching that increases rather than relieves distress.
SHAI-14	14 items, each scored 0–3 (total 0–42); higher = more health anxiety. ≥ 18 = elevated (screening threshold).
CSS-12	12 items scored 1–5 (total 12–60) with four 3-item facets: excessiveness, compulsion, distress, reassurance seeking.
Cross-sectional	Everything measured once, at one time point: shows associations, not cause and effect.
Convenience / snowball sampling	People who were easy to reach, who then shared the link with others. Quick, but not representative.
Mean ± SD	The average score and how spread out the scores are.
Median (IQR)	The middle score and the range of the middle half; used when data are skewed.
p-value	How surprising the result would be if there were truly no effect; $p < 0.05$ is called significant.
95% CI	The range in which the true value probably lies; narrower = more precise.
Correlation r	How closely two scores move together (0 = none, 1 = perfect). 0.47 is moderate.
Partial correlation	The same link after removing the effect of other variables.
Mann–Whitney / Kruskal–Wallis	Non-parametric tests comparing two / three or more groups, used because scores were not normally distributed.
χ^2 test	Tests whether two categorical variables are related (e.g., social-media use and elevated health anxiety).
FDR (q-value)	Correction for running many tests; a result is "robust" if $q < 0.05$, otherwise exploratory.
Linear regression, B and β	Predicts a score from several factors at once. B = change in CSS points per unit; β = the same on a standardised scale (to compare factors).
R^2 and ΔR^2	Share of variation explained by the model; ΔR^2 = extra share explained by adding a block of variables.
Logistic regression, AOR	Predicts a yes/no outcome. Adjusted odds ratio: how many times the odds change, holding other factors constant.
AUC	How well a model separates two groups (0.5 = chance, 1 = perfect).
Mediation	Splitting a link into a part that runs through a third variable (indirect) and the rest (direct). Statistical, not causal.
Moderation	Whether a third variable changes the strength of a link (e.g., does AI use make the link stronger? No).
Cronbach α / McDonald ω	Internal consistency: do the items of a scale hang together? ≥ 0.7 is acceptable, ≥ 0.8 good.
CFA (CFI, RMSEA, SRMR)	Checks the items group into the intended factors. Good: CFI ≈ 0.95+, RMSEA ≤ 0.08, SRMR ≤ 0.08.
Measurement invariance	Checks the scale measures the same thing in two groups (here Arabic vs English) before comparing them.

Sensitivity analysis	Re-running the main analysis in a different way to see if the result holds.

Never say (overclaims the committee will catch):

- "Health anxiety *causes* cyberchondria" → say "is associated with", "is compatible with a cycle".
- "19% *have* health anxiety disorder" → "19% scored in the elevated range on a screening tool".
- "AI makes people anxious" → "we found no reliable independent effect of AI use".
- "Our results represent Baghdad" → "a young, online, convenience sample; associations were consistent across subgroups".
- "The Arabic version is validated" → "we tested it: equal reliability and formal invariance, but no formal back-translation".

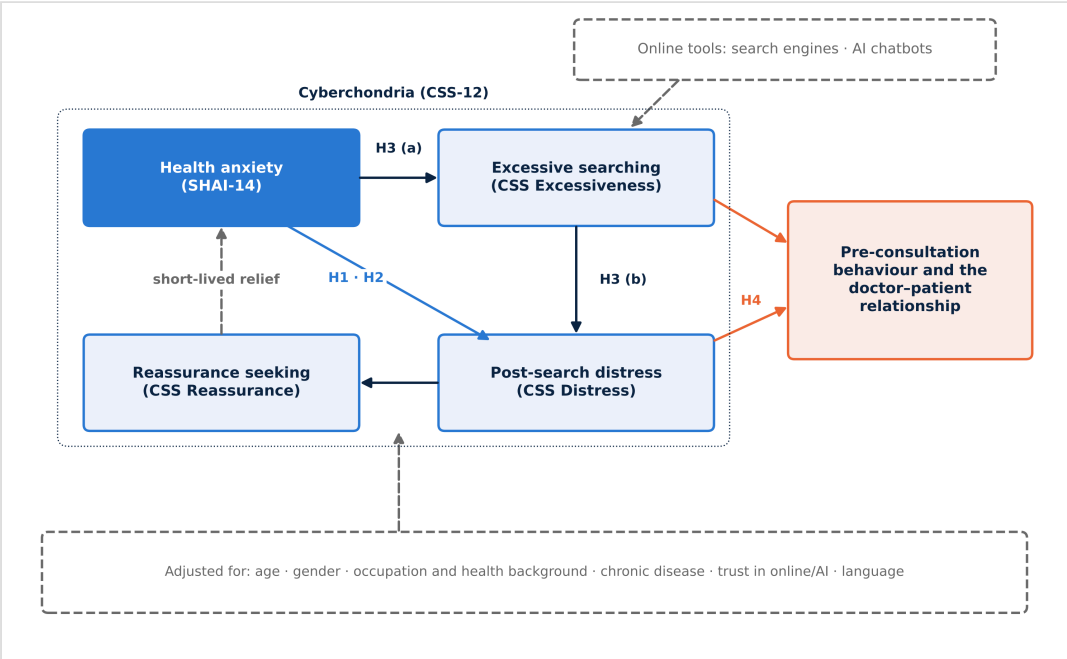


Figure 1 of the thesis: the conceptual framework. Learn to explain every arrow.

Part 2 · Your chapter, section by section

For each section: its **purpose**, the **key points and numbers** (with the reference numbers used in the thesis), and what to **watch**.

1.1 Background

Why the topic matters.

- Searching before seeing a doctor is now ordinary; it helps, but for some people it amplifies worry: **cyberchondria** [1] [2].
- Iraq is highly connected: about **four in five** Iraqis online by 2025 [3], yet nothing had been studied in Baghdad.

Watch: Why Baghdad? Why now? Your answer: high Internet use, rising AI use, no local data.

1.2 Health anxiety

What health anxiety is and why it matters.

- A continuum from normal worry to illness anxiety disorder / somatic symptom disorder (DSM-5) [4].
- Cognitive-behavioural model [5]: sensations misread → anxiety → checking and reassurance → short relief → worry returns.
- Common and costly: ≈ 6% lifetime, up to 20% in hospital outpatients, unnecessary contacts [6][7]; rising over three decades [8].
- Iraq: about half of 1591 online respondents reported health anxiety during COVID-19 [9]; home quarantine raised it in 1578 adults [10].

Watch: Be able to draw the cognitive cycle on a whiteboard.

1.3 Cyberchondria

Definition and measurement.

- White & Horvitz [1]: benign searches escalate to rare, serious diagnoses. Starcevic & Berle [2]: searching that *increases* distress.
- Not pathological in itself; becomes cyberchondria when excessive, compulsive, distressing [11]; possible transdiagnostic compulsive syndrome [12].
- CSS-33 had five factors incl. mistrust of doctors [13]; CSS-12 keeps four [14]; Arabic CSS-12 validated in Lebanon [15].

Watch: Know why we used CSS-12 (short, four facets, Arabic version exists) and that it drops “mistrust”.

1.4 Health anxiety and cyberchondria

Why the two are studied together.

- Related but distinct; cyberchondria adds impairment and healthcare use [16]. Searching raises anxiety in an experiment [17].
- Meta-analyses: r = 0.34 with general online searching but **0.62** with cyberchondria [18]; **0.63**, facets 0.24–0.66 [19].
- Intolerance of uncertainty linked to cyberchondria (r = 0.55) [20].
- Longitudinal: Dutch 4-wave panel of 5322 adults, reciprocal link [21]; pregnancy study, health anxiety as risk factor [22].
- COVID-19 studies in Croatia and Turkey [23][24]; scoping review prevalence 30.7–55.6% [25].

Watch: The meta-analytic 0.62 will come back in the Discussion: our 0.47 is lower. Know that Student 3 explains why.

1.5 From search engines to AI chatbots

What is new about AI.

- LLM chatbots are fluent, personal, repeatable; answers can seem more empathetic than doctors' [26][27].
- US survey: ≈ 1 in 5 used ChatGPT for health; ≈ a third changed medication or asked for referrals [28].

- Health anxiety predicts symptom-checker use [29]; Czech sample: link via trust in AI [30]; Turkish dermatology: 61% used AI before the visit [31].

Watch: Open question you can state: does AI relieve worry (calm answers) or feed it (endless reassurance)?

1.6 Searching and the doctor–patient relationship

Why behaviour matters clinically.

- Discussing searches can help the relationship; hiding them strains it [32][33].
- Arab region: Jeddah [34], Saudi primary care [35], 13 MENA countries: trust in online information predicted delayed consultation [36].

1.7 Rationale

Three gaps.

- First CSS-12 data from Baghdad.
- AI chatbots studied separately from search engines.
- Psychology linked to real pre-consultation behaviour.

Watch: Practise saying the three gaps in one breath.

1.8 Aim, objectives, hypotheses, Figure 1

What the study set out to do.

- Aim: relationship between health anxiety and cyberchondria and their correlates among adults in Baghdad, with attention to AI chatbots and pre-consultation behaviour.
- H1: health anxiety ↔ cyberchondria (mainly distress, excessiveness). H2: independent of other factors. H3: excessive searching mediates HA → distress. H4: cyberchondria ↔ care-seeking and strained relationship.
- H1–H2 primary; H3–H4 secondary; AI comparisons, item-level, moderation and sensitivity analyses exploratory.
- Figure 1 shows these on the cycle.

Watch: Know which hypotheses were supported: H1 yes, H2 yes, H3 statistically (not causally), H4 partially.

2.1 Design and setting

- Analytical cross-sectional; STROBE [37] and CHERRIES [38]; online, Baghdad, 17 July 2026 – 11 September 2026.

2.2 Participants and sampling

- Convenience + snowball via Baghdad-based social-media and messaging groups.
- Inclusion: ≥ 18 years, reads Arabic or English, informed consent.
- All submissions retained; no duplicates; response rate cannot be calculated (snowball).
- Three responses had been annotated by the team: kept, and excluded in a sensitivity analysis.

Watch: Residence in Baghdad was not asked directly: say so honestly.

2.3 Sample size

- Fisher's z: r = 0.20, α 0.05, 80% power → 194 (85 for r = 0.30) [39]. 300 collected; ±5.7% margin for percentages (vs ±5% for 385).
- Logistic model for elevated health anxiety: 57 events / 11 predictors ≈ 5 per variable (limitation).

2.4 Instrument

- Bilingual Google Form, five parts (Figure 2). SHAI-14 [40], validated widely [41][42][43], Arabic version in Saudi Arabia [44].
- CSS-12 [14]: 1–5 scale, four subscales.
- Arabic wording by the team, not formal cross-cultural adaptation [45]; published Arabic versions [15][46][44] not used.
- SHAI item 2 options displayed in a different order; scored by meaning; dropping it changed nothing.

Watch: This is the most attacked section. Prepare the Arabic-version answer (Part 5).

2.5 Variables

- Elevated health anxiety: SHAI ≥ 18 (vs healthy controls, Swedish study) [42]; also 15, 22, 29.
- High cyberchondria: CSS > 33 (upper tertile; no validated cut-off).
- AI user: ticked AI or named a chatbot (32 named without ticking); analyses repeated with the strict definition.
- Health background = medical, other health-science students and healthcare workers.

2.6 Statistical analysis

You name the methods; Student 2 explains them in depth.

- Python 3 (pandas, SciPy, statsmodels, pingouin, factor_analyzer, semopy), fully scripted; SPSS file provided.
- Psychometrics: α, ω [47], CFA by ML (criteria [48]), invariance by multi-group CFA [49] and DIF by ordinal logistic regression [50].
- Correlations; Mann–Whitney/Kruskal–Wallis; χ^2; FDR [52]; hierarchical robust regression; logistic and ordinal; bootstrap mediation [53]; moderation; sensitivity analyses.
- Figure 2 summarises the study flow.

2.7 Ethics

- Voluntary, anonymous, e-consent with age confirmation before any question; no names or contacts.
- [Ethics approval: committee, number, date: fill in before the defence.]

Watch: You must know the approval details by heart.

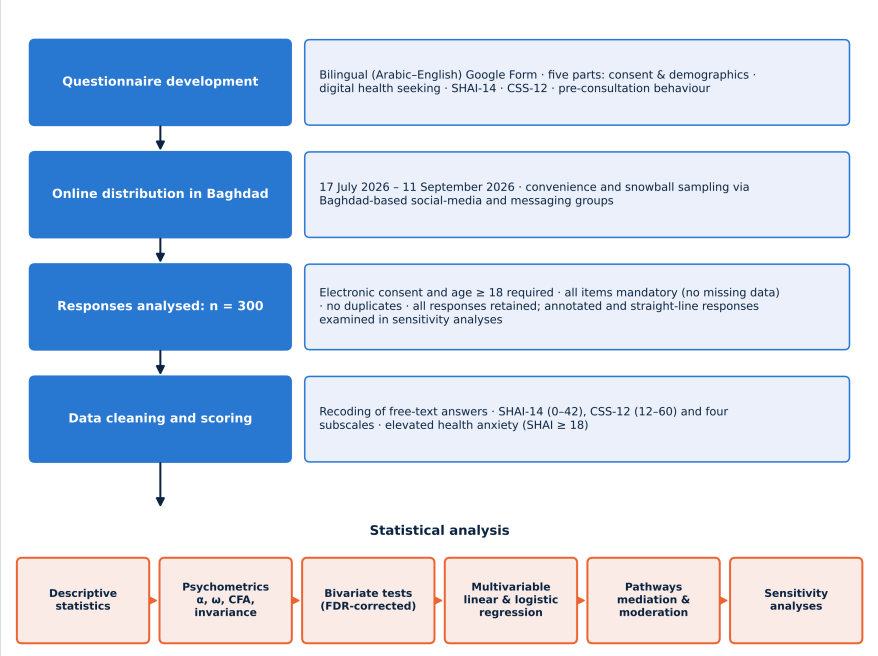


Figure 2: the study flowchart. You present this on the methodology slides.

Part 3 · Your key sources

Every reference cited in your chapter, with its number in the thesis. Know the bold ones well: examiners often ask “who found that?”.

No.	Reference (as listed in Chapter 6)
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Part 4 · Your slides, one by one

The script below is the speaker-notes text of your slides in the presentation. Practise until you can say it in your own words without reading.

Slide 1 · An online cross-sectional survey

Good morning. We are [names], supervised by [supervisor]. Our research is titled: Health anxiety, AI chatbot use and cyberchondria among adults in Baghdad. In simple words: what happens when worried people search their symptoms online, and now ask AI chatbots like ChatGPT, before they see a doctor.

Slide 2 · is my chest pain a heart attack?

We open with two lines. The hadith reminds us that every illness has a cure, so seeking knowledge about health is good. The second line, from Montaigne, describes health anxiety exactly: the fear of illness is itself a kind of suffering. Our study sits between these two ideas: looking for information can help, or it can feed fear.

Slide 3 · Objectives

Our aim was to study the relationship between health anxiety and cyberchondria among adults in Baghdad who search for health information online. We had four objectives. First, describe how people search, including AI chatbots. Second, measure health anxiety and cyberchondria and how strongly they are linked. Third, look for pathways that fit a vicious cycle. Fourth, see how all this affects what people do before and during the consultation.

Slide 4 · Introduction

We will start with the background: what health anxiety is, what cyberchondria is, how they may feed each other, and why AI chatbots change the picture.

Slide 5 · Health Anxiety

Health anxiety is a persistent worry about having a serious illness. It is a spectrum: most people have some health worry; at the far end it becomes illness anxiety disorder or somatic symptom disorder in DSM-5. The cognitive-behavioural model says normal sensations are misread as signs of disease, and safety behaviours such as checking and reassurance seeking give short relief but keep the worry going. It is common and costly. In Iraq, about half of online respondents reported health anxiety during COVID. We measured it with the Short Health Anxiety Inventory, SHAI-14.

Slide 6 · Cyberchondria

The word cyberchondria comes from White and Horvitz, who analysed search logs and found that searches for common, benign symptoms often escalated to rare and serious diagnoses. Starcevic and Berle defined it as excessive or repeated online health searching that increases rather than relieves distress. Searching itself is normal; it becomes cyberchondria when it is excessive, compulsive and distressing. We measured it with the CSS-12, which has four parts: excessiveness, compulsion, distress and reassurance seeking.

Slide 7 · The Vicious Cycle

Why study these together? Meta-analyses show health anxiety correlates strongly with cyberchondria, around r = 0.62. Health-anxious people feel worse after searching, while people with low anxiety feel relieved. A Dutch study of 5322 adults over four waves suggested the relationship goes both ways. This is the cycle on the slide: anxiety drives searching, searching drives distress, distress drives reassurance seeking, and reassurance does not last. We tested whether our data fit this cycle.

Slide 8 · From "Dr Google" to "Dr ChatGPT"

Almost all the evidence comes from the search-engine era. AI chatbots are different: they answer fluently and personally, can be asked again and again, and can sound more empathetic than doctors. In a US survey about one in five people used ChatGPT for health information, and about a third changed medications or asked for referrals based on its answers. No study had compared AI chatbots with search engines in Iraq.

Slide 9 · Methodology

Now the methodology: design, participants, sample size, the questionnaire and the analysis.

Slide 10 · Study Design & Settings

This was an analytical cross-sectional study, reported using the STROBE and CHERRIES checklists. Data were collected online in Baghdad from 17 July 2026 to 11 September 2026. We used convenience and snowball sampling through social media and messaging groups based in Baghdad. Inclusion: 18 or older, able to read Arabic or English, and gave consent. Participation was anonymous and voluntary, with electronic consent before any question.

Slide 11 · Sample Size & Questionnaire

Sample size. Our main question is a correlation. With alpha 0.05 and 80% power we need 194 people to detect a small-to-moderate correlation of 0.20. We collected 300, and the expected correlation from meta-analyses is much bigger, so the study is well powered (power about 94%). The questionnaire was a bilingual Google Form with five parts: consent and demographics; digital health seeking including AI chatbots; the SHAI-14; the CSS-12; and behaviour before the consultation and the relationship with the doctor. Every item was shown in Arabic and English.

Slide 12 · Statistical Analysis

For the analysis we used Python, with every step scripted so it can be reproduced, and we also prepared an SPSS file. We checked reliability and factor structure, then correlations, group comparisons, multivariable regression, and mediation with bootstrapping. Because we ran many tests, we corrected p-values with the false discovery rate method, and we labelled anything that did not survive as exploratory. Now my colleague will present the results.

> **Delivery for your part:** ≈ 5 minutes; about 25–35 seconds per content slide and 10 seconds per divider. Point at the number you are saying. Pause after each key number. End with: "That was our methodology. Now my colleague will present what we found."

Part 5 · Questions and model answers

A. Questions on your part

Q: Why did you choose this topic?

Give your **real** reason (e.g., patients or relatives using ChatGPT before seeing a doctor). Then: no data from Iraq, and AI chatbots had not been studied next to search engines.

Q: What is the difference between health anxiety and cyberchondria?

Health anxiety is the underlying worry about illness; cyberchondria is a behaviour pattern, excessive searching that makes the worry worse. They overlap strongly but are distinct.

Q: Why is your study design cross-sectional?

Feasible for a student project and appropriate for associations. We say clearly it cannot show direction, and we recommend longitudinal studies.

Q: Your sample is not representative. Why convenience sampling?

There is no sampling frame for online adults in Baghdad. We describe the sample honestly, do not generalise percentages, and show associations were consistent across subgroups.

Q: Explain your sample size calculation.

Our primary outcome is a correlation. Fisher's z with α 0.05 and 80% power needs 194 participants to detect r = 0.20. We had 300; meta-analytic r is ≈ 0.62, so the calculation is conservative.

Q: Why not use Cochran's formula (385)?

That is for estimating a proportion with ±5% precision. Our primary aim was a correlation. For percentages, 300 gives ±5.7%, close to ±5%.

Q: Why SHAI-14 and CSS-12?

Both are widely used, short, reliable and have Arabic versions in the literature. CSS-12 has four facets, which let us test the cycle.

Q: Why SHAI ≥ 18?

It best separated people with health anxiety from healthy controls in the Swedish diagnostic study [42]. It is a screening threshold, so we say “elevated”, and Table 4 shows other cut-offs.

Watch out: Never call it a diagnosis.

Q: Did you translate the questionnaire properly?

No formal forward–backward translation, which is a limitation. But we tested equivalence: equal reliability, equal means, full scalar invariance of the CSS-12, metric invariance of the SHAI-14 and no item DIF.

Q: Why didn't you use the published Arabic versions?

We were not aware of them when the form was built [or give your real reason]. We cite them and recommend them for future work.

Watch out: Do not invent a reason; if unsure, say it was an oversight in planning.

Q: How do you know participants lived in Baghdad?

Through Baghdad-based distribution channels; there was no residence question. This is listed as a limitation.

Q: Could one person answer twice?

[Fill in: did Google Forms limit one response per account?] We found no duplicate response patterns.

Q: Why is it called “analytical” cross-sectional?

Because we tested associations and hypotheses, not just described prevalence.

Q: What are STROBE and CHERRIES?

Reporting checklists: STROBE for observational studies, CHERRIES for online surveys. They make sure we report everything a reader needs.

Q: What ethical issues did you consider?

Voluntary participation, anonymity, informed consent before any question, right to withdraw, secure data. [Approval number.]

Q: What are your hypotheses and were they supported?

H1 and H2 supported; H3 supported statistically but not causally; H4 only partially (only disclosure survived FDR).

Q: Why include AI chatbots?

Because they are new and now dominant: 78.0% ticked them. Earlier studies treated "the Internet" as one thing.

Q: What does Figure 1 show?

The conceptual framework: health anxiety, the four facets of cyberchondria on a cycle, the tools that feed searching, covariates, and behaviour as the outcome, with H1–H4 marked.

B. Questions anyone can be asked (know the answer even if it is not your part)

Q: Summarise your study in one minute.

Use the two-minute script in Part 1, shortened: design, sample, two scales, r = 0.47, AI result, behaviour, clinical message.

Q: What is your main finding?

Health anxiety and cyberchondria are moderately and robustly associated (r = 0.47), independent of other factors.

Q: What did you find about AI chatbots?

They are the main source (78.0% ticked), but there was no reliable independent effect on cyberchondria beyond health anxiety.

Q: What percentage had elevated health anxiety?

19% at SHAI ≥ 18, a screening threshold, not a diagnosis.

Q: What is your biggest limitation?

The cross-sectional design: no direction of cause.

Q: What is the clinical message?

Ask patients, without judgement, what they searched or asked an AI.

Q: Why did you split the presentation this way?

By chapter, but each of us studied the whole thesis.

Q: Were your hypotheses written before you collected the data?

Answer truthfully. A safe true answer: they follow from our objectives and previous research, and the questionnaire was designed to measure these things; we tested all of them and report every result, including those not supported.

Watch out: Never claim they were pre-registered.

Q: Who did the statistical analysis?

Answer honestly according to what you did. If AI tools helped with code or drafting, say so in line with your college policy and the declaration in the thesis.

C. Difficult or hostile questions

Q: “This is just a student survey; why should we trust it?”

Calmly: established scales, good reliability, pre-specified hypotheses, correction for multiple testing, robust methods and sensitivity analyses; and we state our limitations openly.

Q: “You found what everyone already knows.”

The link itself is known; what is new is Baghdad data, AI chatbots studied separately, and the link to concrete behaviour before the consultation.

Q: “Why should a doctor care about this?”

Because a third of patients delay care when reassured online and a third hide their searches: both affect diagnosis and trust.

D. Limitations and what was done about each (all students)

Limitation	What we did / how to answer
Cross-sectional design	Reverse-direction models; wording “compatible with”; longitudinal studies recommended
Convenience/snowball sample	Percentages not generalised; associations consistent across subgroups
Arabic wording (no back-translation)	Equal reliability and means; CSS-12 fully invariant, SHAI-14 metric; no item DIF
Single-item behaviour measures	Reported descriptively; associations labelled exploratory
Many tests	FDR correction; robust vs exploratory separated
Small subgroups (34 non-users)	Acknowledged; AI effect called “not robust”
Self-report and careless responding	Anonymous; straight-liners and annotated responses excluded in sensitivity analyses (no change)

> **How to answer any question:** pause → repeat the question briefly → answer in two or three sentences → stop. If you do not know: “We did not examine that directly; based on our data we would expect…, and it is a good question for future research.” Never invent a number. If a teammate struggles: “If I may add…”.

Part 6 · Editing your chapter

> **The Word file is the master copy.** Edit only the .docx, in one shared OneDrive / Word Online file with **Review → Track Changes** on. Keep an untouched copy (Thesis_ORIGINAL.docx). Avoid Google Docs: it breaks the automatic numbering.

For your chapter

- Chapter 1 is the most "human" place to write in your own voice: explain each idea as you would to a classmate, keeping every reference number with its claim.
- Add your genuine reason for choosing the topic (one or two sentences in 1.1 or 1.7) only if it is true.
- In Chapter 2, change wording but never the facts: dates, criteria, sample size numbers, cut-offs, test names, software.
- Write the Acknowledgment yourselves: supervisor, participants, anyone who shared the link, family.
- Fill in 2.7 (ethics approval) and the Google Forms question in 2.2.
- Keep Figures 1 and 2 and their captions; do not type caption numbers by hand.

Never touch

- Numbers, %, p, q, CI, r, OR, β, n
- Reference numbers like [18] (move them with their sentence)
- The reference list
- Table/figure captions, table of contents and lists (automatic fields)
- Heading numbers and styles
- Times New Roman 14 and page numbering

The rewrite method (paragraph by paragraph)

1. Read until you understand it; look up anything unclear.
2. Close it and explain it out loud.
3. Write your version from that explanation.
4. Compare: same facts, numbers and [n]; nothing claimed more strongly.
5. Read aloud; change any phrase you would never say.

After editing

1. Ctrl + A, then F9, to update the table of contents, the lists and the captions.
2. Search for "[" to find placeholders left.
3. Swap chapters with a teammate for checking.
4. Export to PDF and look at every page.

> **About AI use, honestly.** Check your college's AI policy with your supervisor and declare AI assistance if required. A sentence you can adapt: *"The statistical analysis code and an initial draft of this report were prepared with the assistance of an AI tool (Claude, Anthropic). All analyses were checked, and the text was reviewed and rewritten by the authors, who take full responsibility for its content."* Do not use paraphrasing tools or tricks to fool detectors: they make the text worse and can count as misconduct. Real ownership (your words, your understanding, your edit history) is the best protection.

Part 7 · Your 14-day plan

Day	Task
1	Read the Abstract, 1.8 and Chapter 5. Learn the two-minute script (Part 1).
2	Read the whole thesis once, marking what you cannot explain.
3	Core facts and terms (Part 1). Self-test on core facts.
4	Your chapter, sections in Part 2 (first half).
5	Your chapter, second half; your key sources (Part 3).
6	Start rewriting your chapter (Part 6).
7	Finish rewriting. Self-test (Part 8).
8	Swap chapters with a teammate; check accuracy and voice.
9	Your slides: read the scripts (Part 4), then present them alone 3 times.
10	Questions A–D out loud (Part 5). Record yourself once.
11	Team: full rehearsal with timing and handovers.
12	Team: mock committee; each student asks the others their Part 5 questions.
13	Fix weak answers; second full rehearsal; final Word checks.
14	Day before: checklist (last page); one calm run-through; sleep.

Part 8 · Self-test

Cover the answers, answer out loud, then check. Repeat until all are correct.

#	Question	Answer
1	How many participants, and when was data collected?	300; 17 July 2026 – 11 September 2026
2	Required sample size and for what effect?	194 for r = 0.20 (α 0.05, 80% power)
3	Range of the SHAI-14 and CSS-12?	0–42 and 12–60
4	The four CSS-12 facets?	Excessiveness, compulsion, distress, reassurance seeking
5	Cut-off for elevated health anxiety and its source?	SHAI ≥ 18; Swedish diagnostic-accuracy study (Österman 2022)
6	Meta-analytic correlation between health anxiety and cyberchondria?	≈ 0.62 (McMullan 2019) and 0.63 (Schenkel 2021)
7	Who coined “cyberchondria” in search-log research?	White and Horvitz (2009)
8	Inclusion criteria?	≥ 18 years, reads Arabic or English, informed consent
9	Sampling method?	Convenience + snowball through Baghdad-based online groups
10	Which checklists guided reporting?	STROBE and CHERRIES
11	Name the four hypotheses briefly.	H1 HA ↔ CSS; H2 independent; H3 mediation via excessive searching; H4 behaviour/relationship
12	Percentage of Iraqis online?	About four in five (≈ 80%) by 2025
13	How was AI use defined, and why two definitions?	Ticked AI or named a chatbot; 32 named without ticking, so results were repeated with the strict definition

14	Software used?	Python 3 (fully scripted); SPSS file also provided
15	What does the cognitive model say keeps health anxiety going?	Misinterpretation of sensations plus safety behaviours (checking, reassurance) that give short relief
16	Main correlation (everyone)?	r = 0.47
17	Elevated health anxiety (everyone)?	19%
18	AI chatbots ticked (everyone)?	78.0%
19	Never tell the doctor (everyone)?	31.7%
20	Sample size (everyone)?	300

Part 9 · Day-before checklist

✓	Item
☐	All placeholders filled (names, supervisor, college, logo, degree, year, ethics, Google Forms question, Acknowledgment)
☐	Fields updated (Ctrl + A, F9); PDF checked page by page
☐	AI declaration added if required
☐	Presentation: names on slide 1, ethics number on slide 10; USB + PDF + email copies; clicker tested
☐	I can give the two-minute summary and my slides (1–12) without notes
☐	I have answered every question in Part 5 out loud
☐	I scored 100% on the self-test
☐	Printed thesis copy if required; arrive early; agree who stands where

Generated from the study outputs and the presentation, so every number and slide script here matches the thesis and the deck.